



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : صاحبه خليفه نصر حسنين
المحافظة : الجيزه
الحى / المدينة : البدرشين
رقم البطاقة : 27607012105707 رقم التليفون : 01148647227
العنوان : زاويه دهشور
العمر (تاريخ الميلاد) : 01-07-1976 المهنة : ربه منزل
تاريخ تسجيل الإستماره : 20-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مسح ذري علي العظام [12966] قسم العلاج : الاورام
التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.
تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
قسم : -----
جهة العلاج : -----
نفقات : -----
ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: صابحة خليفة نصر حسانين National ID: 27607012105707	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Bone scintigraphy
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



Dms

أفليم
تليودا



MDT FORM

4 / 3 / 2026

G

Name	مها فهد	MRN	214/002
Age	55 years old	ID	
Menopausal status	Postmenopausal	Phone	010-111-0011
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity	HTN DM	REASON OF MDT	

Clinical Data

BX: IDC LTR "Metaplastic"

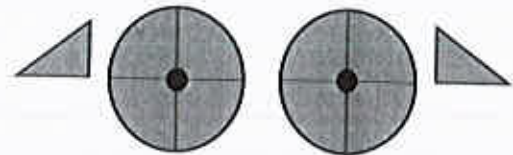
R

L

ER: 018 PR: 2/8 HER 2: - Ki67: 65%

Sonomammography:

Mets Workup:



TNM

Recommendations:

Pr received NACT (DDAC x4 → Taxol / Carb x12)
ending on 10/2/25 → sluggish response from inflammatory
P

MDT OUTCOME & PLAN

Fr.

① Urgent re-stage (CT AP & bone scan).

② Metastatic, shift to Novolac / Xeloda ③ re-challenge by
Carb / Carb if primary
responds.

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
[Signature]	[Signature]				



Patient Name : الأستاذة / صاحبة خليفة نصر حسنين	Request No. : 13008833
Patient No. : 27607012105707	Request Date : 15/10/2024
Path No. : 3933/24	Verification Date : 30/10/2024
Sex : Female	Financial Category : نفقة الدولة - اورام دار السلام
Age : 48 Y	Department : Out Patient
Referring Doc :	Registered by : .marwa mohamed youss

Pathology Report

Clinical Data: Right breast mass.

Nature of Specimen: Tru-cut biopsy, collected at 14/10/2024, 11:00 am.

Gross Examination:

- Three grayish white tissue cores up to 1.2 cm, totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for P63, ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:

- * Histological type: Invasive duct carcinoma (Metaplastic carcinoma).
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+3+2) Grade III.
- * Ductal carcinoma in situ: No.
- * Perineural invasion: No.
- * Angioinvasion: No.
- * Additional pathologic findings: Focal squamoid differentiation.
- * IPT (Positive control showed appropriate reactivity):
 - P63: Focally positive in tumor cells.
 - ER: No nuclear reaction in tumor cells (score 0/8).
 - PR: Weak nuclear reaction in 1% of tumor cells (score 2/8).
 - Her2: No membrane reaction in tumor cells (score 0).
 - Ki67: 60% nuclear reaction of tumor cells.

Diagnosis:

Right breast mass, Tru-cut biopsy:

Invasive duct carcinoma, Grade III (metaplastic carcinoma) with focal squamoid differentiation.

IPT:

Verified by :

Dr. Sagda El-Kady





Patient Name	: الأمستانه / صاحبة خليفة نصر حسنين	Request No.	: 13008833
Patient No.	: 27607012105707	Request Date	: 15/10/2024
Path No.	: 3933/24	Verification Date	: 30/10/2024
Sex	: Female	Financial Category	: أورام دار السلام - نفقة الدولة
Age	: 48 Y	Department	: Out Patient
Referring Doc :		Registered by	: marwa mohamed youss

Pathology Report

- P63: Focally positive in tumor cells.
- ER: Negative (score 0/8).
- PR: Negative (score 2/8).
- Her2: Negative (score 0).
- Ki67: Positive in 60% of tumor cells.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Dr. Rana Fathy.

Dr. Sagda Elkady.

Verified by :

Dr. Sagda El-Kady

Printing Date : 04/11/2024 10:53:04

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Printing By : marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qudema - Cairo, Egypt

PATIENT NAME: Sabahah KHALEEFA NASR HASANEN**GENDER:** Female**DATE :** 19-Nov-2024 05:37 PM**PATIENT ID:** 10124253523**AGE:** 48 y**ORDER:**

3130419,3130579,3130580

Dynamic Contrast Enhanced MRI Study of the Breast**Pulse Sequences:**

- Pre-contrast: axial T1,T2,STIR and sagittal T2 WIs.
- DWI and ADC mapping.
- Post contrast: axial VIBE and Subtraction WIs

****complementary mammography and US were done.****Indication:**

- Pathological right axillary lymph nodes (pathologically proven metastatic of breast origin) to detect any occult breast lesions.

Report:

- Prominent fibroglandular parenchyma of both breasts showing mild nodular back ground parenchymal enhancement.

Right breast:

- Right breast outer quadrants (upper and lower) and deep retroareolar asymmetrical mild heterogenous parenchymal enhancement, more appreciated in delayed phases, corresponding to focal asymmetry on mammography. Complementary US revealed subtle parenchymal alteration with no definite masses.
- Right LOQ small circumscribed oval nodule with intrinsic fat signal measuring about 7x6mm?? IMLN.
- Multiple pathological partially amalgamated right level I,II and III axillary lymph nodes, the largest at level I measuring about 44x31mm. Multiple pathological supra clavicular lymph nodes (seen by US).

Left breast:

- No enhancing lesions are seen, therefore, no time signal intensity curve was measured.
- Left UOQ tiny rounded rim enhancing nodule measuring about 6mm.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Nonspecific axillary lymphadenopathy.

PATIENT: Sabahah KHALEEFA NASR HASANEN, 10124253523, Female, 48 y

Report version 2, 20-Nov-2024 04:02 PM

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مركز بهية للتشخيص
4 شارع علوية اول شارع الهرم بجوار دارى الجيزةمركز بهية للتشخيص
1 الحى الاول الشيخ زايد امام كرة العسمة

well as its main tributaries.

- Gall bladder stone is noted with no peri-cholecystic fluid.
- Left adrenal gland small fat-containing lesion measuring 7mm, likely myelolipoma.
- Normal appearance of the pancreas, spleen (and small adjacent splenules are noted), kidneys (apart from left renal small upper pole cyst), right adrenal, aorta, I.V.C and the pelvic organs.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- No pathologically enlarged abdominal or pelvic lymph nodes.
- No abdominal or pelvic masses or collections.
- **The revised bone window revealed:**
 1. Multilevel vertebral bodies tiny lucent foci.
 2. Dorso-lumbar spondylo-degenerative changes.
 3. No CT evidence of vertebral collapse or extra-osseous soft tissue component.

Opinion:

- **Right breast UOQ few enhancing nodules with pathologically enlarged right axillary as well as supraclavicular lymph nodes. For correlation with sonomammography findings.**
- **No definite pulmonary or hepatic metastatic deposits.**
- **Multilevel vertebral bodies tiny lytic foci for bone scan assessment.**

Reported By,

Toka Elbehairy, MSc

Dr. Mahmoud Ibrahim, MD

18-Nov-2024 01:12 PM

PATIENT: Sabahah KHALEEFA NASR HASANEN, 10124253523, Female, 48 y

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مركز باهية
شارع علوية أول شارع اليوم لحوار داري - الجيزة
م. ش. باهية - شارع باهية - زواجر
1. التي الأول - الشيخ زايد - أمام كلية الهندسة



Women's Health Outreach Program

Name: sabha khalifa nasr

Date: 14-10-2024

Bilateral Full field Digital Mammography

Clinical summary: Long standing history of right breast lump, which is now painful

Technique

Medio-lateral oblique and CC views High resolution ultrasound

Detailed Findings

Right breast

- Heterogeneously dense breast parenchyma (ACR density c).
- No masses, asymmetric densities, distortion or suspicious clusters of microcalcifications.
- Multiple pathologically enlarged axillary lymph nodes
- Ultrasound demonstrated No solid or cystic lesions.
- Normal skin/subcutaneous tissue and nipple/areolar complex.
- Multiple enlarged axillary angulated lymph nodes, all showing eccentric fatty hilum , the largest seen measuring about 4x3 cm.

Left breast

- Heterogeneously dense breast parenchyma (ACR density c).
- No masses, asymmetric densities, distortion or suspicious clusters of microcalcifications.
- Ultrasound demonstrated no solid or cystic lesions.
- Normal skin/subcutaneous tissue and nipple/areolar complex
- Non-specific axillary LNs.

Impression:

Right breast Normal breast study, with suspicious axillary lymph nodes .
(BIRADS 4) for histopathology correlation (biopsy was taken)

Left breast (BIRADS 1): Normal study.

Best regards,
Dr. Norhan Osama , MD

Opinion:

- A case of pathological axillary lymph nodes (pathologically proven metastatic of breast origin) showing right outer quadrants and retroareolar low suspicious parenchymal enhancement for biopsy (BIRADS 4).
- Right LOQ probably benign looking nodule (BIRADS 3).
- Left UOQ probably benign nodule (BIRADS 3).

Reported By,

Dr, Yara Alaa, MSc

Prof.Dr. RASHA KAMAL

20-Nov-2024 04:02 PM

PATIENT: Sabahah KHALEEFA NASR HASANEN, 10124253523, Female, 48 y

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مركز بيه العالم
4 شارع علوية نول شارع الورم بجوار داري , الحجرة

مستشفى بهاء الشيخ زايد
E. الحي الأول - الشيخ زايد أمام كلية الهندسة

PATIENT NAME: Sabahah KHALEEFA NASR HASANEN

GENDER: Female

DATE : 17-Nov-2024 02:30 PM

PATIENT ID: 10124253523

AGE: 48 y

ORDER: 3125992,3125993

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS

TECHNIQUE:

- Multislice helical CT acquisition with automatic IV low osmolar non-ionic contrast medium infusion.

FINDINGS:

1-CHEST

- Right breast UOQ multiple small enhancing nodules; the largest measures 7 mm in diameter.
- Multiple pathologically enlarged right axillary level I, II and fewer level III lymph nodes; the largest is at level I measuring 24 x 34 mm.
- Pathologically enlarged right supraclavicular lymph nodes; the largest measures 15 x 13 mm.
- Tiny right internal mammary lymph node is noted measuring 5 mm.
- Few subcentimetric left axillary level I and II and supraclavicular lymph nodes are noted.
- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitory lesions.
- No pathologically enlarged mediastinal or hilar lymph node.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers.
- No pericardial effusion detected.

2- ABDOMEN AND PELVIS

- Mildly enlarged liver with homogenous CT texture. No focal lesions or intra-hepatic or extra-hepatic biliary radicles dilatation.
- Patent homogeneously enhancing portal vein and its main branches as

PATIENT: Sabahah KHALEEFA NASR HASANEN, 10124253523, Female, 48 y

Report version 2, 18-Nov-2024 01:12 PM

Page 1 of 2



Patient's Name:

Sabha Khalifa Nasr Hassanain

Date:

2026-08-24

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

CLINICAL INDICATION: breast cancer swollen arm.
COMPARISON: No previous study for comparison.
TECHNIQUE: Multiplanar CT with IV contrast neck chest abdomen pelvis.

FINDINGS:

NECK:

The nasopharynx, oropharynx and hypopharynx are preserved, with no obvious mass lesion.

The endolaryngeal structures that include the epiglottis, aryepiglottic folds, false and true vocal cords appear intact.

The laryngeal skeleton that includes the thyroid and cricoid cartilages as well as the hyoid bone appear intact.

The examined salivary glands that include the parotid, sub-mandibular and sub-lingual salivary glands appear intact.

The thyroid gland appears of normal size with no obvious thyroid nodule.

The carotid arteries and internal jugular veins appear intact.

The cervical musculature and visualized osseous structures appear intact.

Right subcentimetric multiple cervical lymphadenopathy.

FINDINGS:

right breast marked skin and subcutaneous thickening and edema involving right breast, visualized part of right neck right chest and abdominal wall.

Right breast is edematous with multiple focal lesions

Multiple right axillary lymph nodes enlargement reaching 3 cm in short axis.

right pectoralis muscle major and minor are seen enlarged edematous, loss of fat planes with heterogeneous enhancement of ill-defined area 4.5 x4 cm.

no evidence of rib involvement or destruction.

TRACHEA: Normal diameter and wall thickness, with no evidence of stenosis.

BRONCHI: Patent, with no evidence of bronchiectasis.

LUNGS: No pulmonary nodules, alveolar infiltration or interstitial thickening.

Right upper lobe posterior pleural thickening and nodularity with ground glass opacity

MEDIASTINUM: No mediastinal mass lesion or lymph node enlargement.

HILUM: right subcentimetric lymph node enlargement.

CARDIAC CHAMBERS:

No chamber dilatation.

PERICARDIUM: No pericardial thickening or effusion.

AORTA (ascending, arch and descending): Normal diameter, with no evidence of aortic aneurysm.

PULMONARY TRUNK AND MAIN PULMONARY ARTERIES: Normal diameter, with no evidence of pulmonary hypertension.

PLEURA: No pleural effusion

Right upper lobe posterior pleural thickening and nodularity with ground glass opacity

CHEST WALL: spondylo-degenerative changes.

Patient's Name: Sabha Khalifa Nasr Hassanain
Date: 2026-08-24
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

LIVER:

Size: average.

Surface: Smooth and regular.

Hepatic focal lesion: No solid or cystic mass lesion.

Portal vein: Patent.

Hepatic biliary system: Not dilated.

GB single large stone

SPLEEN: moderate enlarged 16 cm. No splenic focal lesion.

KIDNEYS: Normal size, parenchymal thickness, density and enhancement. Normal contrast excretion on delayed images. No calculi or back pressure changes on either side. The perinephric fat planes are preserved.

ADRENAL GLANDS: left adrenal shows small 2 hypodense lesions with HU less than 10 HU, likely adenomas

Pancreas: Normal size and density. No pancreatic mass lesion. The pancreatic duct is not dilated.

ABDOMINAL AORTA: Unremarkable. No aneurysmal outpouching or dissection.

IVC: Unremarkable.

LYMPH NODES: No retroperitoneal, mesenteric or pelvic lymphadenopathy.

SMALL AND LARGE BOWEL: duodenal wall thickening is noted
large bowel long segment 8 cm in length proximal to rectum showing luminal narrowing with thickened wall 6 mm.

URINARY BLADDER: increase wall thickness denoting cystitis. No intravesical stone or mass.

RECTUM: Normal wall thickness. The perirectal fascia is preserved.

REPRODUCTIVE SYSTEM: bilateral prominent adnexa right reaching 4 cm.

PERITONEUM: No ascites.

Umbilical hernia passing bowel loop.

BONE: spinal spondylo-degenerative changes.

IMPRESSION:

- A case of right breast cancer with pathological right axillary lymph nodes and invasion of right pectoralis major and minor with no evidence of bone invasion. Right-sided neck, breast, chest and abdominal wall edema. Evaluation of for venous thrombosis is limited on this routine examination. Dedicated venous Doppler ultrasound and/or CT venography is considered.
- Right upper posterior pleural thickening and nodularity. PET CT is recommended.
- Bilateral enlarged ovaries. correlation with transvaginal ultrasound is recommended.
- Segmental bowel wall thickening. clinical correlation and further assessment are recommended.
- GB stone – cystitis -non-complicated umbilical hernia

Dr.Nada Nasser,FEBR

NAME	Sabha Khalifa Nasr	DATE	15-8-2026
RELATION	Dear Colleague		

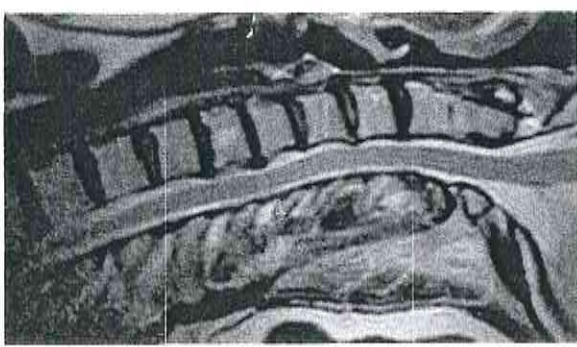
MRI OF THE CERVICAL SPINE

TECHNIQUE:

- Sagittal T1 and T2 WIs
- Axial GE WIs

FINDINGS:

- Straightened cervical curve.
- Normal dimensions of cervical spinal canal.
- Cervical spondylosis evident by small marginal osteophytosis of the vertebral endplates and degenerated discs.
- C3-4 posterior disc protrusion is seen encroaching upon the CSF space and indenting the cord.
- C4-5 mild left posterolateral disc bulge is seen encroaching upon the CSF space and not indenting the cord.
- C5-6 posterior and right posterolateral disc bulge is seen encroaching upon the CSF space and indenting the cord yet partially encroaches upon the right neural exit foramen.
- C6-7 posterior disc bulge is seen encroaching upon the CSF space and indenting the cord yet partially encroaches upon the right neural exit foramen.
- C5-6 and C6-7 bilateral neurocentral arthropathy.
- Normal size and signal of the cervical cord.
- No marrow infiltrations.
- Intact cranio-cervical articulation.
- No para-spinal soft tissue masses or collection.



OPINION:

- Cervical spondylosis with multi-level disc lesions; as described.

Dr. Ahmed Shawky, MD